

2nd week Sunday case scenario

1- dr.

2- dr.

3- dr.

4- dr.

Case 33

Shno is a 27 year old G3P1A1 pregnant of 37wks gestation, presented at 8:00 am on the day of your call to the labour ward with abdominal pain .She is a known case of hypertension on treatment since second trimester. She is a teacher. Family history is positive to hypertension and diabetes mellitus. The family asked you whether she is in labour or not.

33.11 What are the criteria of labor pain to differentiate it from other types of pain?

.....

.....

.....

.....

.....

.....

33:2 After taking full history from Athmar, you performed general, abdominal and pelvic examination. Her vital signs were normal and she was not in labour; however, she was worried about her baby as she is having hypertension. You admitted her for further assessment. How can you assess her fetal condition?

.....

.....

.....

.....

.....

.....
.....

Shno called you at 2:00 pm and informed you that her pain is increasing and she is starting to pass clear watery discharge per vagina.O/E: PR= 88/min, BP=140/90 mmHg, T= 37c. Fetal HR= 130/min. She is having 3 uterine contractions per 10 min, head is engaged. P/V examination: cervical dilatation is 2 cm, cephalic presentation, membranes are ruptured and the liquor is clear.

33.2 At what stage of labour Athmar is?

.....
.....
.....
.....
.....

33.3 Plot these findings on a partogram.

.....
.....
.....
.....
.....

33.4 How would you monitor her fetal condition during labour?

.....
.....
.....
.....
.....
.....

33.5 The patient asked you that she want to eat and drink, what is your advice?

.....
.....
.....
.....
.....

33.6 How frequent you want to perform pelvic examination?

.....
.....
.....
.....

33.7 At 8:00 pm you examined Shno and found that she is in the second stage of labour. She asked you at which position she can deliver. Do you prefer certain position?

.....
.....
.....
.....
.....

33.8 What the meaning of crowning?

.....
.....
.....

33.9 Can you prevent perineal tears at this stage? How?

.....
.....
.....
.....

33.10 Shno delivered a live male baby of 3.250kg body weight spontaneously. At the first min. the delivered baby is pink with blue extremities, his heart rate is 95bpm, and well- flexed limbs, with strong cry and normal respiration. He responded to sole stimulation by a grimace. Shno asked you about the condition of her baby. What is APGAR SCORE of this newborn baby?

.....
.....
.....
.....
.....

33.11 What are the signs of placental separation?

.....
.....
.....
.....

33.12 If the placenta is separated how would you deliver the placenta?

.....
.....
.....
.....
.....

33.13 The placenta was delivered within 10min. Is this labour regarded as normal? What are the criteria of normal labour?

.....
.....
.....

.....
.....
.....

33.14 Give five reasons why a low risk woman in labour might be commenced on electronic fetal monitoring (EFM).

.....
.....
.....
.....
.....

33.15 What four fetal features are assessed when interpreting a cardiotocogram?

.....
.....
.....
.....
.....

33.16 You perform a fetal blood sample (FBS) on the basis of abnormal CTG and the result shows a PH of 7.22. Outline your plane management.

.....
.....
.....
.....
.....